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# Barriers to healthcare for racialised populations in Europe: a scoping review of reviews

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## Abstract

**Background** Racism is a significant barrier to equitable healthcare, yet it remains under-researched in Europe. Barriers across various stages of the healthcare process limit the equitable realisation of the right to health for racialised populations. This review synthesises existing literature on the barriers to healthcare faced by racialised populations in Europe.

**Methods** We conducted a scoping review of reviews to identify barriers to healthcare for racialised populations in Europe. We systematically searched across four databases (PubMed, Scopus, Web of Science, and PsycINFO) for review articles published from January 2010 to September 2024. We obtained data from 30 eligible reviews and classified them according to the dimensions of healthcare access.

**Results** Our review identified multiple, interrelated barriers across the dimensions of availability, accessibility, acceptability, and quality. Key barriers include insufficient interpretation and intercultural mediation services, complex and restrictive administrative procedures, economic costs, and cultural insensitivity in clinical practices. Racialised individuals also reported experiencing interpersonal racism and stereotypes, which affected their healthcare experiences and outcomes. The review highlights a significant lack of cultural competence among healthcare professionals, as well as inadequate adaptation of services to meet the needs of diverse populations. Additionally, we identified legal and administrative barriers, particularly for undocumented migrants, as crucial obstacles.

**Conclusions** Our findings underscore the need for systemic changes to address the structural racism that limits equitable access to European healthcare systems. There is a critical need for racism-reflexive healthcare, with culturally competent healthcare services, improved training for healthcare professionals, and resources adapted to the needs of a diverse population. Additionally, research should focus on the experiences of racialised populations in under-researched settings, such as primary care, as well as on racialised populations beyond the most vulnerable. This review calls for health policies that ensure the effective exercise of the right to health for all individuals, regardless of their racial or ethnic background.

**Keywords** Racialised populations, Healthcare barriers, Europe, Access to healthcare, Structural racism, Cultural competence, Migrant health

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**Resumen (español)**

**Antecedentes** El racismo constituye una barrera significativa para lograr una atención sanitaria equitativa, estando aún insuficientemente investigado en Europa. Las barreras a lo largo de varias etapas del proceso de atención sanitaria limitan la realización equitativa del derecho a la salud para las poblaciones racializadas. Esta revisión sintetiza la literatura existente sobre las barreras al acceso a la atención sanitaria que enfrentan las poblaciones racializadas en Europa.

**Métodos** Se realizó una revisión de alcance de revisiones para identificar las barreras al acceso a la atención sanitaria para las poblaciones racializadas en Europa. Se buscaron sistemáticamente revisiones en cuatro bases de datos (PubMed, Scopus, Web of Science y PsycINFO) para artículos publicados entre enero de 2010 y septiembre de 2024. Se extrajo información de 30 revisiones elegibles, las cuales se clasificaron según las dimensiones del acceso a la atención sanitaria.

**Resultados** La revisión identificó múltiples barreras interrelacionadas en las dimensiones de disponibilidad, accesibilidad, aceptabilidad y calidad. Las principales barreras incluyen servicios insuficientes de interpretación y mediación intercultural, procedimientos administrativos complejos y restrictivos, costes económicos e insensibilidad cultural en las prácticas clínicas. Las personas racializadas también informaron haber experimentado racismo interpersonal y estereotipos, lo que afectó sus experiencias y resultados en la atención sanitaria. La revisión resalta la falta significativa de competencia cultural entre los profesionales sanitarios, así como una inadecuada adaptación de los servicios para satisfacer las necesidades de las poblaciones diversas. Además, se identificaron barreras legales y administrativas, especialmente para los migrantes indocumentados, como obstáculos cruciales.

**Conclusiones** Los hallazgos de esta revisión destacan la necesidad de un cambio sistémico en los sistemas de atención sanitaria europeos para abordar el racismo estructural que limita el acceso equitativo a la atención. Existe una necesidad urgente de una atención sanitaria sensibilizada hacia las cuestiones raciales, con servicios de salud culturalmente competentes, mejor formación para los profesionales sanitarios y recursos adaptados a las necesidades de una población diversa. Además, la investigación debe centrarse en las experiencias de las poblaciones racializadas en contextos insuficientemente investigados, como la atención primaria, así como en las poblaciones racializadas más allá de las más vulnerables. Esta revisión aboga por políticas de salud que aseguren el ejercicio efectivo del derecho a la salud para todas las personas, independientemente de su origen racial o étnico.

**Palabras Clave** Poblaciones racializadas, Barreras de acceso a la atención sanitaria, Europa, Acceso a la atención sanitaria, Racismo estructural, Competencia cultural, Salud de los migrantes

## Introduction

The right to health is a fundamental human right. From a broad approach to the determinants of health, realising this right entails both ensuring adequate living conditions that enable a healthy life and guaranteeing timely and adequate access to healthcare [1]. Thus, access to healthcare means having the opportunity to reach and obtain services that are appropriate to people's health needs [2]. Although equitable and high-quality access to healthcare for the entire population is a stated commitment of European countries, inequalities persist even in health systems considered universal [3]. One group that faces particularly marked inequities in access to healthcare due to their devalued position in European societies is racialised populations, who encounter multiple types of barriers at different stages and levels throughout their interaction with the healthcare system [4, 5]. For this reason, European healthcare systems, beyond maintaining institutional commitments [6] and moving towards more *migrant-sensitive healthcare* [7], need to broaden their focus to understand how racialisation affects migrants, asylum seekers, refugees, and other racialised groups, and how these processes translate into access-related inequities.

Research in Europe has focused on identifying access barriers faced by migrant, asylum-seeking and refugee populations, especially from legal and administrative perspectives [7]. However, apart from the UK, less attention has been paid to the barriers arising during healthcare provision itself, as a consequence of the racialisation processes affecting these populations and their implications in terms of structural subordination [8]. In much of continental Europe, the historical legacy of colonialism, along with the trauma of the Nazi Holocaust, has contributed to the invisibilisation of existing racial hierarchies in society and to a generalised denial of racism [9]. However, invisibilisation of race and racism has ceased to make sense. Past essentialist notions of race, which were employed by white supremacists to justify racial health inequalities through biological and genetic explanations, are increasingly challenged in epidemiological research [10]. Meanwhile sociological perspectives that view race as a social construct enable the attribution of racial health inequalities to the embodiment of cumulative

exposure to differential social and environmental experiences derived from structural racism [11].

As Jones defines it, "racism is a system of structuring opportunity and assigning value based on the social interpretation of how one looks (which is what we call 'race') that unfairly disadvantages some individuals and communities, unfairly advantages other individuals and communities, and saps the strength of the whole society through the waste of human resources" [12]. Racism can thus be understood as a structural system of social stratification, embedded within the economic, political, and cultural institutions of society, whereby groups are categorized and hierarchically ordered through historically rooted processes of racialisation [8]. Racialisation involves a process of othering which provides the symbolic and discursive mechanisms through which racialized groups are constructed as fundamentally different or deviant [13]. These processes are mutually reinforcing: the social interpretation of physical appearance that underpins racism relies on pre-existing narratives of difference, deviance, and cultural incompatibility. Consequently, racism and othering operate together to produce and reproduce social exclusion, health inequities, and institutional marginalization, ultimately creating barriers to healthcare access.

The European blindness to racism also poses a challenge for its study, as the concept of race is not recognised as a valid category for scientific analysis of inequalities, whether quantitatively or qualitatively, in most European contexts. Meanwhile, historical forms of racism persist and adapt to new migrant populations, maintaining structures of exclusion and inequality. In this context, the category of migrant functions as an implicit racial marker and has frequently been used to study processes of racialisation, highlighting how power structures and social narratives construct and perpetuate racial differences based on migration status [14]. Although the notion of race must be understood as a social construct, that is, a historical and dynamic category that varies across social contexts [15], it nevertheless continues to function as a category of social stratification. Racism in healthcare systems constitutes a fundamental barrier to equitable care, with wide-ranging consequences across health outcomes and access indicators, as extensively documented in diverse national settings [4, 16, 17].

The study of racism within healthcare in Europe remains not only extremely scarce [4] but also rarely looks beyond interpersonal racism [18]. Research on inequity in healthcare systems tends to focus on the most vulnerable migrant populations—such as undocumented migrants or refugees—highlighting the challenges they face in accessing care. Moreover, these challenges are often analysed from an individualising or even blaming perspective, attributing difficulties to personal characteristics such as lack of language proficiency, socioeconomic vulnerability, or irregular legal status [3]. This analytical lens risks decontextualising access to healthcare systems, shifting attention away from the broader structural and institutional barriers embedded in the organisation and functioning of healthcare systems or in the practices of healthcare professionals. As scholars such as Lupton [19] and Fassin [20] have argued, public health and healthcare research often reinforce a model of individual responsibility that obscures these broader determinants, contributing to a depoliticised and moralising framing of vulnerability. To overcome this limitation, Metzl and Hansen [21] propose the concept of 'structural competency', which calls for healthcare professionals and researchers to develop the ability to recognize and address the social, economic, and political forces shaping health outcomes. Yet, less attention is given tothe capacity of European healthcare systems to meet the needs of diverse populations, or to the effect of of implicit and explicit racial bias [22] that extend beyond the most vulnerable migrant groups. This highlights the need to identify existing knowledge gaps in order to broaden the focus from a *migrant-sensitive* approach to a truly *racism-reflexive* in the design and delivery of healthcare systems. Identifying barriers to healthcare—drawing on the dimensions set out in models by Tanahashi [23] and Levesque et al. [2] — contributes to a better understanding of the complex ways racialisation may intersect with the right to health for migrant, asylum-seeking, and refugee populations, as well as for those with a minority racial or ethnic background in Europe. This review therefore aims to identify and categorise the barriers to healthcare faced by racialised populations in Europe.

## Methods

### Study design, search strategy and eligibility criteria

We designed this scoping review with a descriptive approach based on reviews examining barriers to healthcare for racialised populations in Europe. The review was registered in OSF (DOI: https://doi.org/10.17605/OSF.IO/76DQ3). A search was conducted across four electronic databases—PubMed, Scopus, Web of Science, and PsycINFO—for reviews published between 1 January 2010 and 30 September 2024.

To define the racialised population, we applied the concept of the "race–migration nexus" proposed by Erel et al. [14], which highlights how racialisation shapes the construction and stratification of migrant populations through exclusionary practices based on place of birth and citizenship status. This broad definition was used as a search criterion to circumvent the reluctance to use racial and ethnic categories in studies conducted in European countries and to move beyond a narrow focus

on migratory status as the sole determinant of access to healthcare.

The inclusion and exclusion criteria are outlined in Table 1. Studies conducted in the UK were excluded due to differences in how racialised populations are defined and measured compared to continental Europe, given the UK's Anglo-Saxon tradition of considering race and ethnicity as analytical categories. Moreover, to fulfil the objective of examining the available evidence and identifying knowledge gaps regarding barriers to healthcare access for racialised populations in Europe, the large volume of literature from the UK would have dominated the sample and hindered adequate identification and assessment of findings relevant to other countries. Similarly, studies focusing exclusively on Roma or Indigenous populations were excluded, as the processes of othering affecting these groups—while sharing some similarities—are shaped by distinct historical, socio-political, and epistemological contexts. We consider that the literature on these populations deserves dedicated analysis beyond the scope of this review. In addition, reviews or articles that focused solely on healthcare professionals' perceptions of barriers were excluded, as our aim was to synthesise evidence from the perspective of racialised individuals navigating the healthcare system.

The search strategy (Box 1, PubMed) combined text words and Medical Subject Headings (MeSH) to identify literature related to healthcare systems, access barriers, racialised populations and the European context. The same search strategy was used across all four databases, with adaptations specific to each platform. The search was carried out using the following fields: "abstract" (in Web of Science and PsycINFO), "Article title, Abstract, Keywords" (in Scopus), and "All Fields" (in MEDLINE). Further details are provided in the Protocol for Search Strategy and Data Extraction, which has been published elsewhere [24].

**Box. 1** Search strategy for PubMed

("Health Services"[Mesh] OR healthcare OR "health care" OR "health system" OR "Primary Care" OR "Primary Health Care"[Mesh] OR hospital OR "Emergency Medical Services"[Mesh] OR screening OR Testing) AND (Acceptability OR Accessibility OR Availability OR Affordability OR barrier* OR access* OR experiences) AND (racism OR racial* OR "Refugees"[Mesh] OR "asylum seek*" OR migrat* OR migrant* OR immigra* OR emigrant* OR "displaced people" OR "foreign born" OR "foreign population" OR "Non-national" OR "Transients and Migrants"[Mesh] OR "Ethnic and Racial Minorities"[Mesh] OR "ethnic minorit*" OR "ethnic group" OR ethnicity) AND (Europe OR "European Region" OR "European Union" OR "European Economic Area" OR "EU/EEA" OR "European Free Trade Association" OR EFTA OR Euro OR Albania OR Andorra OR Austria OR Belarus OR Belgium OR Bosnia OR Herzegovina OR Bulgaria OR Croatia OR Cyprus OR "Czech Republic" OR Denmark OR Estonia OR Finland OR France OR Germany OR Greece OR Hungary OR Iceland OR Ireland OR Italy OR Latvia OR Liechtenstein OR Lithuania OR Luxembourg OR Malta OR Monaco OR Moldova OR Montenegro OR Netherlands OR "North Macedonia" OR Norway OR Poland OR Portugal OR Romania OR San Marino OR Serbia OR Slovakia OR Slovenia OR Spain OR Sweden OR Switzerland OR Ukraine OR "High-income countries" OR "Holy See").

### Study selection and data extraction

Three reviewers independently assessed the relevance of all identified reviews to the study objective. During the title and abstract screening phase, reviews were excluded if they focused solely on studies conducted in the UK or outside of Europe, if they analysed only inequalities in the use of healthcare services, if they addressed only the perspective of healthcare professionals, or if their objectives were deemed irrelevant to the aim of this review. During full-text screening, additional exclusions were

**Table 1** Inclusion and exclusion criteria

| Criteria | Inclusion Criteria | Exclusion criteria |
|---|---|---|
| Study Focus | Barriers to healthcare<br>Impacts of racism/racial discrimination on access to the healthcare system | Inequalities in healthcare use<br>Impacts of racism/racial discrimination on health |
| Population /Perspective | Racialised populations: migrants by country of birth/nationality, asylum seekers, refugees, non-White racial groups, non-majority ethnic groups<br>Children, youth, or adults | Roma populations<br>Indigenous populations<br>Health professionals' perspectives |
| Geographical location | EU27 countries<br>Non-EU Europe: Albania, Andorra, Belarus, Bosnia-Herzegovina, Iceland, Liechtenstein, Monaco, Moldova, Montenegro, North Macedonia, Norway, San Marino, Serbia, Switzerland, Ukraine, Holy See. | Non-European context<br>Non-Europe: UK and countries not listed in the inclusion criteria. |
| Setting | All levels of care and types of health promotion or prevention programmes: Primary care, Specialised care, Emergency services, Hospitalisation, Vaccination, Screening programmes, Health promotion programmes. | Nursing homes, long-term care services, community-based health, and social care services |
| Research design | Have used systematic search processes: Systematic reviews (with or without meta-analysis), scoping reviews, integrative reviews, systematic integrative reviews, meta-ethnography reviews, and narrative reviews. | Primary studies (quantitative or qualitative)<br>Umbrella review, reviews of reviews or scoping reviews that include other reviews. |
| Language | Articles published in English, Spanish or French | |
| Publication period | From 1 January 2010 to 30 September 2024 | |

applied to reviews that analysed only inequalities, those without any studies conducted in European countries, or those including fewer than three relevant studies—either initially or after removing studies already covered in the other included reviews. The software Rayyan [25] was used to support the article selection process. Further details are provided in the Protocol for Search Strategy and Data Extraction [24].

### Data synthesis

From each of the selected reviews, information was extracted on the methodology used (Table 2): first author, publication period covered, type of review, number of studies included (total and those conducted in European countries), type of studies and/or populations excluded, included populations, and healthcare service/setting/intervention. Subsequently, we identified barriers to healthcare and grouped them according to key dimensions drawn from established models of healthcare access. Three models served as references for selecting these dimensions. First, the framework of the UN Committee on Economic, Social and Cultural Rights (2000) defines four essential elements of the right to health: availability, accessibility (including physical, economic, and informational), acceptability, and quality. Second, Tanahashi's model [23] outlines several stages in healthcare provision, asserting that services must be available, accessible, acceptable, and provided on a continuous basis and with good quality. Finally, Levesque et al. [2] identify five dimensions: approachability, acceptability, availability and accommodation, affordability, and appropriateness. Based on these three models—and considering the specific characteristics of racialised populations—we selected seven dimensions to classify the barriers to healthcare reported in the reviews: availability, physical accessibility, administrative accessibility, economic accessibility, informational accessibility, acceptability, and appropriateness.

In reviews that included studies beyond the selected European countries, we considered only data relevant to European countries. For reviews where both racialised populations and healthcare professionals were included as sources of information, we extracted only findings related to racialised populations.

## Results

After removing duplicates found across the four databases, we identified a total of 3,221 reviews. Title and abstract screening resulted in 65 articles being selected for full-text review. In the end, 30 articles met the eligibility criteria. The selected reviews covered 469 records; 397 unique primary studies remained after duplicate removal. Figure 1 presents the flowchart outlining the inclusion and exclusion process for the reviews.

Table 2 summarises the main characteristics of the reviews analysed. Most of them were systematic reviews (16), followed by scoping reviews (11), with other types being less common, such as one integrative review, one narrative review, and one meta-ethnography review. The reviews covered wide time periods, most frequently including studies published between 2000 and 2022. The countries mentioned in the reviews were Sweden (60), Spain (48), Norway (45), Germany (44), Netherlands (40), Italy (33), Switzerland (28), France (25), Denmark (20), Belgium (22), Portugal (16), Finland (15), Greece (8), Poland (3), Ireland (2), Czech Republic (1), Hungary(1), Malta(1), Croatia(1), Romania(1), Slovakia (1) and one study covering EU27 countries.

Regarding the description of study populations, the vast majority of reviews focused their analysis on migrant populations, using country of birth as the main defining variable. Only two reviews used the term "racialised population" [4, 18], and three reviews included the term "ethnic minorities" [35, 36, 52], although mostly in relation to UK-based studies, which were excluded from our results. Two studies also included descendants of migrants [30, 31]. Five reviews focused exclusively on undocumented migrants [33, 34, 39, 46, 53], while three others specifically included this group among the populations studied [32, 38, 42]. Five reviews focused on refugees and asylum seekers [37, 40, 41, 44, 50], while eight additional reviews also included these groups specifically [26, 27, 32, 36, 38, 42, 45, 49].

Eleven reviews analysed barriers to healthcare access exclusively for women—whether undocumented, refugees, or belonging to racial or ethnic minorities [28, 30, 32, 33, 37, 41, 45–48, 52]. Six of these women-focused reviews specifically addressed prenatal care (including mental health) [30, 32, 37, 41, 46, 52], and other two addressed sexual and reproductive health of women [28, 48], and one of undocumented migrants [39]. Five reviews focused on mental health services [35, 36, 41, 45, 50] and four more referred to HIV testing and counselling [26, 27, 29, 47]. Notably, seventeen reviews examined barriers to these specific healthcare services, while eight reviews examined barriers to the healthcare system in general, without specifying any particular service, setting, or intervention [4, 18, 33, 38, 40, 44, 53]. Other reviews explored evidence on barriers related to vaccination [31], cervical cancer screening [42], hepatitis B screening [43], palliative care [51], or in emergency care [34].

The main findings from the 30 included reviews are presented according to the dimensions of analysis described in the methods section. Table 3 summarises the barriers analysed in relation to each of the following dimensions: availability, physical accessibility, administrative accessibility, economic accessibility, informational accessibility, acceptability, and appropriateness.

**Table 2** Description of reviews included

| First author, year publication | Years searched | Type of review | Nº of studies/Nº of studies in Europe. European countries (Nº) | Types of studies and/or populations excluded in review | Population included | Health service/Setting/Intervention |
|---|---|---|---|---|---|---|
| Adrián Parra 2024 [26] | 2000–2022 | Scoping review | 44/10<br>Belgium (1), Ireland (1), Norway (3), Portugal (1), Spain (2), Sweden (2) | Internal migration, tourists, migrant children, maternal healthcare (prenatal, delivery, and postpartum) | Migrants, Refugees, Asylum seekers, aged > 15 years | Sexual and reproductive health; HIV and other Sexual Transmission Infections testing and/or counselling |
| Álvarez-del Arco 2013 [27] | 2005–2009 | Systematic review | 37/3<br>France (1), Spain (2) | Not mentioned | Migrants, Ethnic groups, transients refugees | HIV testing and/or counselling |
| Barrio-Ruiz 2024 [28] | -2022 | Integrative review | 19/9<br>Denmark (1), Norway (3) Finland (1), Spain (1), Sweden (1), Switzerland (2) | Internal migration, views of men or health professional's | Migrant women, aged ≥ 18 years | Sexual and reproductive health |
| Blondell 2015 [29] | 1997–2014 | Systematic review | 31/9<br>Belgium (2), Ireland (1) Netherlands (2), Portugal (2), Spain (2) | Migrant men who have sex with men, female sex workers, and/or injecting drug users, intervention studies, non-peer-reviewed articles | First generation migrants (from lower middle-income countries World Bank classif.) | HIV testing and/or counselling |
| Boerleider 2013 [30] | 1995–2012 | Systematic review | 16/7<br>Netherlands (2), Switzerland (2), Sweden (2), Italy (1) | Illegal immigrants, refugees, asylum seekers, students or migrant farmworkers, systematic literature reviews, non peer-reviewed articles | 'Non-western' women (from Turkey, Africa, Latin-America, Asia) and descendants | Prenatal care |
| Crawshaw 2022 [31] | 2000–2021 | Systematic review | 43/31<br>Belgium (1), Denmark (4), France (3), Germany (5), Greece (1), Greece and Netherlands (1), Hungary (1), Italy (5), Netherlands (4), Poland (1), Portugal (1), Spain (1), Sweden (3) | Migrant status not defined by country of birth or not defined, data not collected from one of the listed countries data, no disaggregation between migrants and non-migrants; Non-original research articles. | Adult, adolescent (foreign-born) and children of migrants residing in Europe | Vaccination uptake |
| Fair 2020 [32] | 2007–2017 | Systematic review | 51/30<br>Czech Republic (1), France (1), Finland (2),<br>Germany (1), Greece (1), Ireland (3), Italy (1), Netherlands (3), Norway (5), Sweden (6), Switzerland (1), Portugal (4), Spain (1) | Internal migrants; migration status unclear; systematic reviews and reviews; non peer-reviewed articles; women's experiences of interventions | First-generation migrant, refugees, asylum seekers, undocumented women | Prenatal care |
| Gea-Sánchez 2017 [33] | 2004–2014 | Scoping review | 16/16<br>Spain (16) | Not mentioned | Undocumented immigrant women | Healthcare (no concretion of services, settings or interventions) |
| Hamed 2022 [4] | -2020 | Scoping review | 213 /15<br>Belgium (3), Finland (2), France (3), Ireland (1), Sweden, Portugal and Germany (1), Romania (1), Slovakia (1), Spain (1), Sweden (1) | Grey literature, non-empirical articles | Racialized minorities | Healthcare (no concretion of services, settings or interventions) |
| Kisa 2024 [34] | -2024 | Scoping review | 12/5<br>Denmark (1), Norway (2), Spain (1), Switzerland (1) | Non-empirical articles | Undocumented migrants | Emergency care |
| Kjøllesdal 2023 [35] | -2022 | Scoping review | 8/4<br>Denmark (1), Five European countries (1), Germany (1), Norway (1), | Indigenous people, children and adolescent, general population (not patients/users) | Migrants, ethnic minority, aged ≥ 16 years | Mental healthcare |

**Table 2** (continued)

| First author, year publication | Years searched | Type of review | Nº of studies/Nº of studies in Europe. European countries (Nº) | Types of studies and/or populations excluded in review | Population included | Health service/Setting/Intervention |
|---|---|---|---|---|---|---|
| Krystallidou 2024 [36] | 2011–2022 | Systematic review | 58/17<br>Denmark (1), 16 European countries (1), Italy (1), Germany (5), Netherlands (1), Norway (3), Spain (1), Sweden (1), Switzerland (3) | Non peer-reviewed articles | Migrants, refugees, asylum seekers, displaced persons, stateless people, ethnic minorities | Mental healthcare |
| Leppälä 2020 [37] | 2013–2018 | Systematic integrative review | 10/10<br>Finland (2), Norway (3), Sweden (5) | Not-qualitative design, or the phenomenon of interest focusing not specifically in maternity care | Refugee and asylum-seeking women | Prenatal care |
| Luiking 2019 [38] | 2000–2016 | Meta-Ethnography review | 27/7<br>Denmark (1), Netherlands (1), Norway (1), Poland (1), Sweden (3) | Children and maternity care, non qualitative research designs, systematic reviews, non peer-reviewed articles. | Documented and undocumented migrants, refugees. | Healthcare (no concretion of services, settings or interventions) |
| Mandroiu 2024 [39] | 2017–2024 | Systematic review | 37/28<br>Belgium (2), Denmark (3), Finland (1), France (4), Germany (1), Italy (4), Netherlands (1), Norway (1), Sweden (3), Portugal (1), Spain (6), Switzerland (1) | Non peer-reviewed articles | Undocumented migrants | Sexual and reproductive health |
| Mangrio 2017 [40] | – 2017 | Scoping review | 26/4<br>Greece (1) Netherlands (1), Sweden (2), | Only views of health professionals, no differentiate gender quotas nor discuss females | Asylum seekers, refugees | Healthcare (no concretion of services, settings, or interventions) |
| Markey 2024 [41] | 2010–2023 | Scoping review | 18/6<br>Germany (3), Norway (1), Sweden (1), Switzerland (1) | Studies that no differentiate between migrant subgroups, even if they included refugees and asylum seekers | Refugee and asylum-seeking women within the WHO European Region | Perinatal mental healthcare |
| Marques 2020 [42] | – 2019 | Scoping review | 20/18<br>Denmark (1), Finland (2), Italy (3) Norway (4), Poland (1), Spain (3), Sweden (4) | Grey literature | Documented and undocumented migrants, asylum seekers, refugees. | Cervical cancer screening |
| Moonen 2023 [43] | 2015–2022 | Systematic review | 68/49<br>Belgium (2), Denmark (1), France (7), France and Italy (1), Germany (3), Greece (2), Italy (15), Netherlands (6), Norway (1), Spain (6), Sweden (2), Switzerland (1), Malta (1), Greece and Netherlands (1) | Epidemiological or microbiological focus, non peer-reviewed articles | Migrants | Hepatitis B screening |
| Nowak 2022 [44] | 1992–2021 | Scoping review | 44/38<br>Austria (1), Denmark (1), European countries (4) Germany (13), Ireland (1), Italy (1), Netherlands (5), Norway (1), Sweden (5), Switzerland (6) | Pregnant refugees, internally displaced people, grey literature, systematic reviews, individual case analyses, diagnostic and therapeutic studies | Refugees, asylum seekers, aged > 18 years | Healthcare (no concretion of services, settings or interventions) |
| Nyikavranda 2023 [45] | – 2021 | Systematic review | 19/10<br>Austria (1), Netherlands (1), Norway (2), Finland (1), Germany (1), 8 European countries (1), Sweden (2), Switzerland (1) | Grey literature | Migrants, asylum seekers, or refugees women | Primary care; Mental healthcare |

**Table 2** (continued)

| First author, year publication | Years searched | Type of review | Nº of studies/Nº of studies in Europe. European countries (Nº) | Types of studies and/or populations excluded in review | Population included | Health service/Setting/Intervention |
|---|---|---|---|---|---|---|
| Osuide 2024 [46] | Not mentioned | Systematic review | 12/11<br>Belgium (1), Denmark (3), France (2), Finland (1), Norway (2), Sweden (2) | Documented migrants and undocumented migrants outside Europe, non peer-reviewed articles, case studies, intervention research | Undocumented migrant pregnant women | Prenatal care |
| Owusu 2023 [47] | 2011–2021 | Systematic review | 12/9<br>Various countries (4), Denmark (1), Netherlands (2), Spain (1), Sweden (1) | Not mentioned | Migrant women (cis and trans) | HIV testing and/or counselling |
| Pattillo 2023 [18] | 1992–2022 | Scoping review | 38/30<br>Belgium (4), Finland (1), France (3), Germany (2), Ireland (1), Netherlands (2), Norway (5), Portugal (1), Spain (2), Sweden (5), Switzerland (3), ten European countries (1) | Non-empirical articles, non–peer- reviewed articles, a non-migrant or unclear study population, a different study focus, grey literature, systematic and non-systematic reviews | Racialized migrants | Healthcare (no concretion of services, settings, or interventions) |
| Pérez-Sánchez 2024 [48] | 2018–2023 | Systematic review | 28/8<br>Belgium (1), Germany (1), Greece, Italy and Spain (1), Norway (1), Sweden (1), Switzerland (3) | Studies based on health professional interviews and research focused on child health, women with specific characteristics or pathologies, systematic reviews | Migrant women | Sexual and reproductive health |
| Rade 2018 [49] | 2000–2017 | Systematic review | 15/5<br>Belgium (1), Ireland (1), Spain (1), Sweden (2) | Undocumented migrants; aged ≤ 18, countries that had no Universal Health Coverage systems; Healthcare providers perspectives; non-peer reviewed articles | Migrants, refugees | Sexual and reproductive health |
| Satinsky 2019 [50] | 2007–2017 | Systematic review | 27/21<br>Croatia (1), Germany (3), Denmark (3), 14 European countries (1), 7 European countries (1), Germany (3), Ireland (2), Italy (1), Netherlands (2), Norway (4), Sweden (2), Switzerland (2) | Undocumented migrants, other types of immigrants or marginalized populations | Refugees or asylum seekers | Mental healthcare |
| Shabnam 2022 [51] | 2011–2020 | Systematic review | 35/16<br>Austria (1), Belgium (3), Denmark (1), Germany (3), Netherlands (4), Norway (2), Sweden (2) | unclear ethnic background of the migrants<br>Non-empirical and non-peer reviewed articles | Non- western migrants, aged > 18 years | Palliative care |
| Sharma 2023 [52] | 2010–2021 | Systematic review | 30/18<br>Belgium (1), Denmark (1), Finland (2), Germany (1), Netherlands (1), Norway (1), Portugal (5), Spain (1), Sweden (3), Switzerland (2) | Care during the Covid-19 pandemic; antenatal screening tests, diagnosis of antenatal diseases, grey literature, non peer-reviewed articles, professional or father perspectives | Ethnic minority women | Prenatal care |
| Suess 2014 [53] | 2009–2012 | Narrative review | 27/25<br>EU27 (6), various countries (19) | Not mentioned | Undocumented migrants | Healthcare (no concretion of services, settings, or interventions) |

### Availability

One of the barriers identified in healthcare for racialised populations is the availability of interpretation services. Although such services are increasingly widespread across many European countries, several reviews report either their absence in certain healthcare settings [28, 44, 46, 50] or that they are insufficient [26, 49] or lacking competence, particularly in the use of medical terminology [28, 44, 49]. All of this creates difficulties throughout the process of interaction with healthcare services, from navigating access to diagnosis and/or treatment, including the actual delivery of care. A lack of cultural mediation services is also identified [31, 33, 39], which

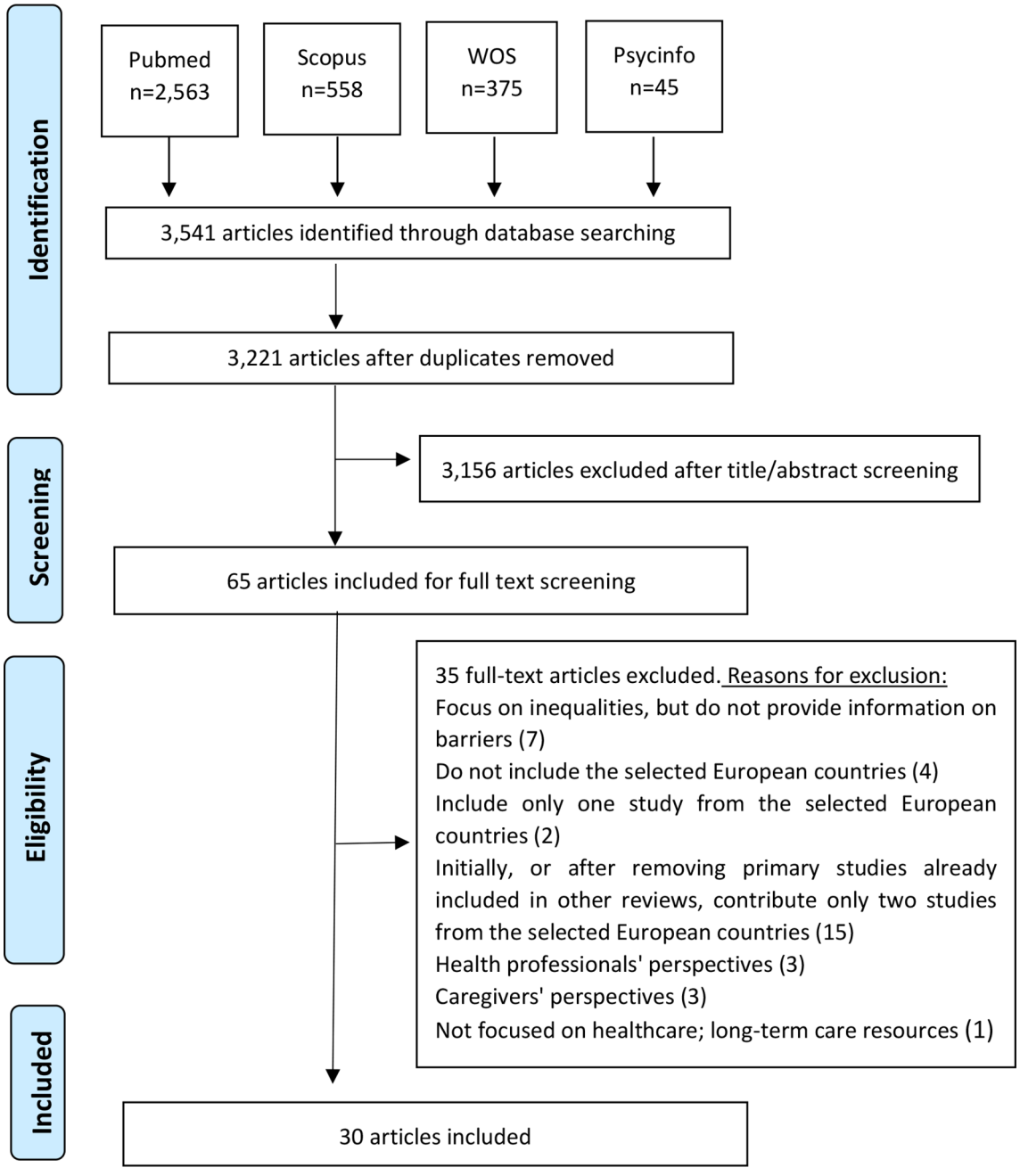


**Fig. 1** Flow chart of retained and excluded reviews

could help facilitate engagement with healthcare services beyond purely linguistic barriers.

### Physical accessibility

The reviews identify barriers related to physical accessibility, referring to the distance to healthcare services, transportation links, and the opening hours of healthcare facilities. Some reviews highlight a need for extended and flexible opening hours [32, 43], as restrictive schedules are often incompatible with the work or caregiving responsibilities of racialised people, thus acting as a barrier to healthcare [26, 30, 50]. Other reviews mention waiting times as a barrier to access [29,44], which may lead to delays in undergoing tests or screening procedures [42, 43]. In some cases, wait times are perceived as comparatively longer than those experienced by the non-racialised population [18]. In addition, for undocumented migrants, the distance to both specialised services and those offered free of charge can act as a deterrent to seeking healthcare [39].

**Table 3** Barriers identified according to dimensions

| Dimensions | Barriers identified |
|---|---|
| Availability | Insufficient and/or incompetent interpreting services [26, 28, 44, 46, 49, 50]<br>Lack of cultural mediation services [31, 33, 39] |
| Physical accessibility | Inflexible appointment hours or incompatible with patients' work or caregiving activities [26, 30, 32,43, 50,52]<br>Long waiting times [18, 29, 42–44]<br>Long distances to healthcare facilities [39] |
| Administrative accessibility | Restrictive legislation for access among undocumented migrants and asylum seekers [26, 33, 34, 38, 39, 47, 48]<br>Lack of national or regional guidelines for undocumented migrants [29, 39, 48]<br>Lack of knowledge among healthcare and administrative professionals about migrants' health rights [27, 31, 32, 53]<br>Denial of the right to healthcare [37, 46, 53]<br>Lenghty or complex administrative procedures [34, 39, 43, 52, 53]<br>Difficulties in booking appointments, due to the digital divide or similar [32, 36, 37, 52] |
| Economic accessibility | Direct costs for services not covered or only partially covered [26, 29, 31, 34, 39]<br>Direct cost of medications [49, 53]<br>Indirect costs (transport to health centres, inability to take time off work or obtain sick leave) [26, 31, 32, 39, 40, 49, 50, 52] |
| Informational accessibility | Difficulty navigating the healthcare system [26, 30, 32, 33, 41, 46, 49, 51, 52]<br>Lack of information about the right to healthcare and available services [31, 32, 50–53, 33, 38, 40, 42, 45, 46, 48, 49]<br>Language barriers when seeking information [31, 41–43, 45, 48]<br>Lack of adaptation of medical terminology [28, 32, 37, 43] |
| Acceptability | Lack of cultural sensitivity from professionals in responding to patients' needs [27, 29, 37, 41]<br>Differences in the construction of health concepts and their determinants [26, 28, 29, 32, 33, 36–38, 41–44, 47, 48, 50–52]<br>Stereotyping of racialised people [18, 26, 28, 32, 41, 44, 45, 49]<br>Patients' demands not identified or not taken seriously [4, 18, 29, 32, 37, 40, 42]<br>Negative attitudes, disrespectful behaviour, lack of empathy [32, 36, 37, 40–42, 47]<br>Fear of rejection or racial discrimination by healthcare professionals [4, 26, 29, 31, 32, 37, 44, 47, 48]<br>Fear of being reported or deported [29, 31, 32, 37, 39, 43, 46, 53]<br>Lack of trust in the confidentiality of information [26, 36, 43]<br>Language barriers in patient–provider communication [26, 28–30, 33, 36–38, 40, 41, 44–49, 51, 52] |
| Appropiateness | Services not adapted to the needs of the population [26, 28, 31, 35, 37, 39, 41, 42]<br>Lack of adapted informational materials [31,43, 44]<br>Deficits in healthcare professionals' cultural competence training [28, 37, 39, 41, 43, 51, 53]<br>Lack of continuity of care [39]<br>No inclusion from preventive or health promotion services [31, 39, 43, 53] |

### Administrative accessibility

Within the dimension of administrative accessibility, the first and most significant barrier is the existence of restrictive legislation regarding access to healthcare for migrants with irregular administrative status [26, 33, 34, 38, 39, 48], as well as for asylum seekers [44]. Some reviews also point to regulations that are unclear or subject to frequent change, or to the absence of national or regional guidelines on the right to access HIV testing [29, 47] or sexual and reproductive health services [39, 48]. Healthcare professionals and administrative staff are not always aware of the healthcare entitlements of migrant populations [27, 31, 32, 53], which in practice results in an administrative barrier. There are also documented cases in which the right to healthcare is denied even when existing legislation provides for it [37, 46, 53].

Among migrants whose right to healthcare is formally recognised, difficulties in accessing care are also reported due to administrative procedures, which are often slow and complex [34, 39, 43, 53], especially when individuals are unfamiliar with the host country's language or with how the healthcare system operates. Finally, difficulty in booking appointments [36, 52], in some cases related to the requirement to use telephone or online booking systems [32, 37] is also identified as an administrative barrier.

### Economic accessibilility

We also identified economic barriers in the European literature, related to both direct and indirect healthcare costs that may prevent people from seeking care. Among the direct costs, reviews mention the cost of medical consultations when people lack public health coverage [32, 34], including in universal and publicly funded healthcare systems. This barrier is particularly salient in the case of undocumented populations, for whom it may be difficult to prove residence or meet the required length of stay in the country [39]. In addition, even when public healthcare coverage is available, some services are either not included or require co-payments [26, 29]. This often applies to sexual and reproductive healthcare, such as contraceptive treatments or HIV diagnostic tests [26, 29, 49], as well as to vaccinations [31], and the cost of medications, which may also be subject to co-payment [49, 53]. All of these factors represent obstacles to adhering to prescribed treatments.

On the other hand, the indirect costs associated with healthcare services also constitute *de facto* barriers to accessing care. This includes the cost of transportation to healthcare facilities [26, 32, 49, 50, 52], especially when the only services available to undocumented migrants are located at considerable distances [39]. Additional barriers include the cost of taking time off work—whether a full day or a few hours—to attend a medical appointment or of taking sick leave, particularly for those facing economic and job insecurity [26, 31, 49]. These factors may ultimately result in patients not attending medical appointments at all [32, 40].

#### Informational accessibility

One of the most frequently identified barriers to access is the absence of essential knowledge needed for effective access to healthcare. Reviews often highlight a lack of basic orientation information required for navigating the healthcare system [26, 31, 41, 49, 52], due to limited knowledge of how the system is organised—often very different from that of migrants' countries of origin [30, 33, 51]. To achieve effective healthcare coverage, information must be available in an appropriate language and format [31, 32, 42, 48], enabling individuals to understand what healthcare resources are available and how to access them. Several reviews also identify a lack of information regarding the right to healthcare itself [38, 40, 52] as well as about the services available [33, 45, 46, 49, 50, 53].

In some cases, limited access to information is linked to language-related difficulties, as information is often provided only in the language of the host country [31, 41, 42]. Even when people have a general command of the language, reviews highlight a lack of familiarity with medical terminology [28, 32, 37] or difficulties with reading [43, 45] as barriers. These language-related challenges are worsened by limited awareness of the availability of language support services [28, 48].

#### Acceptability

Barriers related to acceptability in healthcare provision account for a large proportion of the difficulties reported in the reviews we analysed, giving us more material to synthesize and retain more detail themes. Migrant populations describe a lack of cultural sensitivity among healthcare professionals providing care [27, 29, 37, 41]. In particular, reviews highlight issues concerning differences in the sociocultural construction of concepts of health and illness. In some communities, health and illness are interpreted through more holistic approaches, which may also incorporate religious beliefs and the idea that illness is linked to sacred will. Such views contrast with the predominant biomedical models in Europe [26, 33]. These differing understandings of health can lead to misunderstandings and misdiagnoses [36, 38]. In addition, certain beliefs (for instance, cancer-related fatalism, differing symbolic understandings of the body, and the associated stigma within their own communities) may limit participation in diagnostic testing, preventive screenings, or vaccination [28, 29, 31, 48]. Sometimes, the information provided is perceived as incompatible with cultural beliefs and practices related to pregnancy and childbirth [37]. Cultural, religious and social practices also shape how people engage with the healthcare system [32]. Women from countries where prevention is not commonly practiced tend to seek care only in the case of illness, which limits access to preventive services such as cancer screenings or prenatal care [28, 42, 52]. Differing expectations around the postpartum period have also led to the absence of help-seeking for perinatal mental health [41].

Mutual lack of understanding between patient and provider on the causes and management of certain illnesses, such as dementia or HIV, also reflects gaps in acceptability. For example, both unfamiliarity with diseases that are more prevalent in Europe [44], and limited awareness of the importance of early diagnosis, especially when no visible symptoms are present, are common in some migrant communities[47]. Religion, in turn, can act both as a barrier and a facilitator in relation to healthcare, influencing decisions around screening or treatment [43, 47]. Reviews also report differences in expectations regarding disease management and treatment. For example, different expectations may arise for the case of mental health [50], the use of medication [49], or how a diagnosis should be communicated and the doctor–patient relationship handled [51].

Reviews also identify various examples of stereotyping and *othering* of racialised people by professionals, which affect the care process and influence the quality of healthcare provided. Through processes of *othering*, a sense of distance is created between professionals and patients, who are treated as a homogeneous group based on their migration background [18], or refugee status [41]. In many cases, professionals make assumptions—rooted in prejudices linked to perceptions about the social and cultural characteristics of patients' countries of origin—that these patients have a low socioeconomic and educational level [28, 44]. As a result, healthcare professionals often focus their attention on cultural or psychological aspects, potentially overlooking serious health conditions [32]. These biases tend to deny racialised patients' credibility, rationality, and responsibility in the management of their own health [18].

Prejudices also lead to assumptions about the fraudulent use of healthcare resources [45]. In addition, the stereotypical association of certain illnesses with specific communities is common—for example, linking HIV to the African community, thereby perpetuating stigma

against this population [26, 49]. These types of stereotypes affect not only the perceptions of healthcare professionals, but also interactions with other patients, as migrants from African countries are often viewed as carriers of infectious diseases in host countries [18].

These stereotypes contribute to the experience of both implicit and explicit biases in racialised patient interactions with healthcare professionals and can discourage them from engaging with healthcare services. Several reviews show that racialised patients feel their health concerns are not taken seriously and that their symptoms and complaints are not adequately considered [4, 18, 29, 32, 37], that medical examinations were insufficient [18, 40], or that they were treated differently compared to non-racialised patients [18]. Reviews also report inadequate explanations regarding diagnosis and treatment [37, 40, 42]. Similarly, some reviews identify more explicit biases in interactions with racialised patients, which are expressed through negative attitudes such as disrespectful behaviour, rudeness, anger, lack of empathy or warmth during consultations, disinterest, or emotional detachment [18, 28, 32, 36, 37, 40–42, 47].

These phenomena contribute to increased fear of rejection or racial discrimination among racialised communities. Several reviews across different types of healthcare services show that the fear of being judged and subjected to racial prejudice acts as a barrier [4, 26, 31, 37, 47, 48]. This fear applies both to being rejected by healthcare professionals [29] and to the experience of admission or entry into healthcare facilities [32]. One review focused on refugee healthcare reported that experiences of hostility made individuals feel like a burden on the healthcare system [44].

Among undocumented migrants and asylum seekers, fear of deportation is a major concern. This often inhibits seeking care due to fears that the health system may report them to the police or immigration authorities [29, 31, 32, 37, 46], leading in many cases to delayed diagnosis or even to the absence of antenatal care [39]. In some cases, this fear is linked to the diagnosis itself (such as HIV), and the potential consequences it may have for obtaining a residence permit or citizenship [39]. In other cases, fear of deportation acts in the opposite direction, as a facilitator for screening for communicable diseases, when such tests are conducted in detention centres and considered mandatory for asylum applications [43]. Likewise, in refugee reception camps, frequent and organised campaigns tend to result in higher vaccination rates [31]. Deportation fears are closely linked to a lack of trust in the confidentiality of the information and personal data provided to the healthcare system or shared with healthcare professionals [26, 36, 43].

Finally, within the dimension of acceptability, one of the key barriers we identified relates to linguistic difficulties in communication between patients and healthcare professionals [26, 28, 30, 33, 37, 41, 44, 45, 49]. Beyond the language barriers that racialised people face throughout the entire process of accessing healthcare, significant communication difficulties persist during consultations and in the course of care provision itself, affecting the overall quality of care.

These barriers often result in one-way communication, where patients are unable to express their needs or ask questions, leading to dissatisfaction and a sense of distance from healthcare professionals [28, 51]. They may also prevent adequate explanation of diagnostic procedures or treatments [46]. Poor communication can hinder the collection of accurate symptom information and cause misunderstandings of culturally shaped expressions of illness, potentially resulting in misdiagnosis [48], particularly around mental health [36]. Furthermore, difficulties in understanding instructions and interacting with professionals may reduce the use of preventive services [29, 47] or perinatal care [37, 40, 46, 52]. These challenges not only impact racialised patients but also affect healthcare professionals, increasing their sense of uncertainty during care and further contributing to the distancing from patients [38, 51]. However, language barriers are not fully resolved by the presence of interpreters. Some reviews report that patients perceive a lack of trust and confidentiality with interpreters [29, 37, 44], as well as discomfort, especially when discussing sensitive topics or receiving a serious diagnosis [51].

#### Appropriateness

The dimension of appropriateness is related to the adequacy of services to ensure the provision of high-quality care. In this sense, reviews describe barriers due to a lack of adaptation to the needs of the population, as healthcare services are not culturally competent or sensitive to cultural diversity [26, 31, 37, 41]. This misalignment is particularly evident in specific services such as sexual and reproductive healthcare [39, 42]. For example, the lack of female doctors can be a barrier to cervical cancer screening for migrant women, for whom being examined by a male doctor is not acceptable due to cultural or religious values [42]. Inadequate standards of care have also been found in perinatal services [39]. Similarly, in relation to mental health, racialised populations report that care does not meet their needs; for example, being placed in group therapy with members of the host population, despite not speaking the language or having different ways of understanding illness and treatment [35].

Reviews also note that translated materials or information adapted for people with limited reading comprehension in the host country's language are not always available [44], for example in the cases of screening invitations [43] or information about vaccines [31]. In

addition, the reviews highlight a significant lack of training in cultural competence among healthcare professionals to provide appropriate care for these populations [28, 37, 39, 41, 43, 51, 53].

A lack of continuity in care is another sort of barrier that undermines the quality of care [39], and which is sometimes due to poor coordination within healthcare systems. This can be linked to frequent changes of residence or to shifts in entitlements to healthcare that migrants pass through, particularly for the case of undocumented migrants [39]. In addition, undocumented migrants are sometimes not included in preventive services [53], such as cancer screening [39] or vaccination campaigns [31].

### Barriers that particularly affect women

Although several barriers affecting racialised women have already been mentioned, we here outlines those identified by the reviews as being particularly relevant. One such case involves the care of women who may have experienced sexual violence during the migration process and who, upon arrival in Europe, require urgent gynecological care, which is often unavailable [39]. The same applies to women who are victims of gender-based violence or trafficking, whose access to care is left to the discretion of the attending professional, particularly in situations of irregular legal status and lack of entitlement to healthcare [39].

Cases have also been reported in which perinatal care was denied to the undocumented status of pregnant migrant women [46]. Likewise, barriers in the care of women arise due to the lack of female healthcare professionals, particularly related to perinatal, sexual or reproductive health. For such cases, female professionals can facilitate interaction and comfort [26, 41, 42]. A lack of female interpreters may similarly cause discomfort [26].

## Discussion

Our findings highlight diverse barriers to healthcare experienced by racialised populations in Europe, occurring throughout the entire healthcare process and across various dimensions. Barriers refer both to difficulties in accessing necessary resources, such as interpretation or intercultural mediation services, as well as challenges in obtaining care at times and in geographical locations that accommodate patients' constraints. Many barriers arise at the point of access and stem from complex, lengthy, restrictive, and costly administrative procedures. Receiving effective and high-quality healthcare is further hindered by the presence of racial and cultural stereotypes and processes of *othering* by clinical practitioners, which may intersect with gender biases, thus affecting the acceptability of healthcare services. Additionally, we identify a lack of alignment between healthcare services and the needs of racialised populations for the provision of quality care. This includes issues such as the absence of adapted information, insufficient cultural competence training for healthcare professionals, and the non-inclusion of racialised populations in health promotion and prevention programmes. Together, these interrelated barriers form a pattern of systemic inequality that shapes how care is accessed, experienced, and delivered. They reveal that access to healthcare services in Europe remains far from equitable for racialised populations.

Our findings are consistent with those of two other reviews focused on European countries [54, 55]. Driedger et al. [54], specifically examine the accessibility and acceptability of screening, vaccination, and treatment of infectious diseases among migrants. They highlight significant barriers related to the cost of care, the complexity of administrative procedures, and difficulties navigating the healthcare system. The review also emphasises the role of sociocultural differences in beliefs about health and the value of prevention between migrant populations and the host society. These sociocultural constructions of health are among the most salient barriers identified in our review, as they can reduce participation in diagnostic testing, preventive programmes, and vaccination.

Lebano et al. [55]performed a review of primary studies, systematic reviews, and grey literature, examining access to healthcare for refugees, asylum seekers, and immigrants. In line with our findings, they highlight the presence of administrative barriers, which are particularly restrictive in the case of undocumented migrants. Even when legal entitlement to healthcare exists, additional barriers arise in navigating the system, largely due to a lack of information about patients' rights and the services available. Notably, and in consistent with our review, they underscore language barriers that hinder access to perinatal care services, particularly due to the shortage of interpreters and cultural mediators.

Finally, the review by Egli-Gany et al. [56], which included (but was not limited to) European countries across all the studies reviewed, reports similar structural barriers that prevent migrant and refugee populations from maintaining good sexual and reproductive health, with a particular focus on sexually transmitted infections, sexual violence, and unintended pregnancies.

European countries recognise, under normative frameworks and ethical principles, the right to healthcare for all individuals [57], including migrants, refugees, and asylum seekers [6]. More recently, this right has also been extended to all those with a minority racial or ethnic background [58]. However, all European countries have regulations that directly or indirectly limit the healthcare rights of racialised individuals, establishing, under legal and administrative criteria, who is entitled to rights [7].

In any case, even given the right to healthcare, the healthcare system, like any social institution, actively produces and reproduces structural racism through significant barriers arising from the organisational setup of health systems and their limited permeability, flexibility, and adequacy to the needs of diverse populations. A paradigmatic example of this gap between a right and its effective realisation can be found in language barriers. While most healthcare systems in Europe provide interpreting services, our findings highlight that, in many cases, these services are lacking, insufficient, or inadequate. As a result, communication barriers persist when interacting with healthcare services or seeking health information.

Factors related to physical or economic accessibility such as waiting times, co-payments, travel distances, or lost wages, highlight the need to rethink inequalities in access within European systems. These barriers interact with socio-economic and power inequalities, increasing the health vulnerability of racialised populations. Therefore, the barriers faced by racialised individuals in their interaction with the healthcare system are not only significant obstacles to accessing healthcare services but also reproduce and reinforce the beliefs and values of a healthcare system that is focused on a hegemonic population, thus rendering racialised populations invisible or *de facto* denying them access.

At the same time, implicit forms of structural racism also permeate healthcare systems through their professionals. One of the barriers to effective and high-quality healthcare for racialised individuals is related to manifestations of interpersonal racism during interactions—not only in clinical interactions but also with administrative services. Based on our findings, racialised groups explicitly highlight receiving both implicit and explicit biases in interpersonal relationships, as also evidenced in other healthcare contexts [22]. These biased interactions produce feelings of being ignored, stereotyped, and receiving lower-quality healthcare services. Such situations are the product of a healthcare system that seems to perpetuate a bias, whether unconscious or not, about who matters and who deserves respectful, high-quality treatment. All of this contributes to amplifying exclusion, reducing access to services, limiting participation in decision-making, and result in worse health outcomes for racialised populations [4, 5].

Our review shows that one of the most common barriers faced by racialised individuals is related to differences in the sociocultural construction of ideas about health and its determinants, leading to difficulties in understanding between professionals and patients. The hegemonic biomedical model in Europe contrasts with the different social representations of health and the causes of diseases held by non-hegemonic populations [59, 60]. These differences, when they are neither recognised, understood, nor accepted by the healthcare system and, especially, by healthcare professionals, have significant detrimental effects on the prevention and treatment of diseases. Therefore, it is essential to create awareness and training in academic and educational environments to strengthen healthcare professionals' cultural competence [61], as well as structural competence [21] to provide appropriate responses to diversity.

Finally, in terms of health services, settings, or interventions, special attention should be given to the fact that most of the studies included in this review focus on sexual and reproductive health services, HIV testing and counselling, Hepatitis B, cervical cancer screening, and mental health. Among the reviews we analysed, few examine barriers to primary care services. In this regard, a recent review also highlights that the literature pays less attention to non-communicable diseases and preventive services [55]. These services are also linked to populations that are particularly vulnerable, specifically women, undocumented individuals, refugees, and asylum seekers, as well as children. In contrast, studies examining the barriers and experiences of the majority of racialised individuals, migrants, or those with a minority racial or ethnic background who are not in the most socially disadvantaged situations or do not have specific health needs are much less frequent.

This leads us to consider that the focus of research on the experiences of racialised populations in European healthcare systems is particularly centred on how access is facilitated to healthcare services that reduce health risks associated with the arrival of migrant populations in host societies. To some extent, there seems to be greater interest in the adverse consequences for the host populations of health problems experienced by migrant populations in more vulnerable situations, rather than ensuring the effective exercise of the right to health for all.

In this sense, beyond focusing solely on migrant populations, we adopted a broader approach that emphasises racialisation—acknowledging the race–migration nexus, while also moving beyond it. Given the lack of clear definitions and the diverse ways in which the literature refers to these populations, our search strategy included terms such as ethnic minorities, ethnic groups, and ethnicity when related to colonialism or to ethnic background. Our work builds upon the contributions of previous reviews by Hamed et al. [4] and Pattillo et al. [18], as both adopt the concept of racialised populations and help broaden the analytical focus on how racialisation shapes access to healthcare. Specifically, Hamed et al. provide a global overview of the topic, highlighting the limited use of such terminology in continental Europe. Pattillo et al., while also employing a racialisation lens and focusing

specifically on the European context, remain largely within the analytical boundaries of migrant populations.

Furthermore, as we hypothesised at the beginning of the review, research on the healthcare system in Europe is still colorblind [62, 63]. Building on the concept of *color-evasiveness* developed in the U.S. context [64], recent scholarship suggests that European societies remain embedded in a form of *race-evasiveness*—a deeper reluctance to acknowledge race and racism even when addressing structural inequalities [65]. In Europe, racism has shifted from a model based on the supposed biological inferiority of certain groups to a form of "neo-racism" grounded in their presumed cultural incompatibility with national identity. This evolution has given rise to more subtle expressions of racism, which operate through categories such as nationality, ethnicity, and immigration status, and continue to legitimize the exclusion and stigmatization of those perceived as a threat [66]. Although some research initiatives in different European countries have addressed racism in various areas of life, including experiences in healthcare [67, 68], a race-informed approach within public health and health services research is still lacking, limiting meaningful progress on this issue.

### Strengths and limitations

To the best of our knowledge, this is the first study that synthesises the barriers faced by racialised populations in accessing different healthcare services in European countries. Our scoping review approach, based on the systematic identification of reviews and the exhaustive selection of results specific to the continental European context, has allowed us to extract relevant information from almost 400 primary studies, covering diverse methodological approaches, varying definitions of the racialised population or the type of service, and different languages of publication. Furthermore, our findings contribute to increasing knowledge on how racialisation operates in healthcare access, which is necessary to address both our understanding of and the visibility of racism in Europe.

The methodological variability among the reviews analyzed—particularly in terms of eligibility criteria and the level of thoroughness with which the findings of primary studies were individually synthesized—limits the possibility of making direct comparisons between reviews, and even more so between primary studies. However, it does appear possible to note that the higher frequency of certain reported barriers over others seems to be more closely related to the characteristics of the populations studied (e.g., undocumented migrants, refugees, or women) and to the healthcare settings analysed (such as prenatal care, mental health, or HIV testing), rather than to methodological differences. When these factors are taken into account, a convergence in the identification of barriers can be observed, even among studies with different approaches. This consistency across reviews, despite methodological heterogeneity, highlights the need for context-sensitive approaches in addressing healthcare access inequities.

However, our review presents several limitations. First, we did not conduct a critical appraisal of the quality of evidence from the included reviews, which is optional in scoping reviews [69]. Rather, our primary aim was to map the available literature with a descriptive focus. Second, it was challenging to achieve comparability in the results due to the diversity of healthcare systems and the specific legislative and social contexts of the European countries analysed. Additionally, there may be substantial differences in the lived experiences depending on the country of origin, the migration or refugee profile, or the racial or ethnic background of the populations studied. Consistent with the aforementioned *race-evasiveness*, our results primarily refer to the barriers faced by migrants, refugees, or asylum seekers, and we still know little about what is happening with racial or ethnic background populations. All these factors may limit the representativeness and generalisability of the findings to other countries, geographical contexts, or racialised subpopulations. Another limitation of this review is the uneven geographical representation of the included studies. Western European countries, such as Spain, Sweden, Norway, and Germany, dominate in both number and diversity of health services and migrant populations studied. In contrast, Eastern European countries (e.g., Slovakia, Hungary, Romania) are underrepresented. This may reflect lower research output on migration and health in these settings, as well as the exclusion of non-English publications. Nevertheless, the barriers identified in these few studies are broadly consistent with those reported elsewhere. Finally, since this review is based on reviews rather than primary studies, despite the large number of primary studies covered by the reviews we review and the consistency in the results, it is possible that relevant primary studies with disparate results were excluded.

## Conclusions

In Europe, multiple barriers that limit equitable access to healthcare for racialised populations persist. Throughout all stages of healthcare, racialised individuals, coming from structurally devalued positions, face various difficulties in terms of availability, accessibility, acceptability, and quality, all of which restrict the effective exercise of the right to health. The lack of adaptation in the design and organisation of healthcare systems to the diverse populations they serve, combined with the reproduction of racial stereotypes and biases in clinical practice, reinforces exclusionary dynamics that affect not only the most vulnerable migrant populations (undocumented,

refugees, or asylum seekers) but also all racialised groups. Thus, barriers to access and quality care are not only related to resources or infrastructure but also to mutual understanding and respect for cultural differences.

Despite the normative recognition of the right to health without discrimination, in practice, European healthcare systems continue operating under a model that prioritises the needs of the hegemonic population, ignoring racial inequalities and the impacts of structural racism on healthcare access and delivery. The findings of our review confront us with this critical challenge that must be addressed without delay. This challenge is heightened in a context where, on one hand, neoliberal influences and the treatment of health as a commercial commodity have exacerbated processes of healthcare exclusion, impacting both the functioning and funding of healthcare systems [70]. On the other hand, the rise of far-right ideologies that fuel hate speech against racialised populations poses significant risks, undermining progress toward eliminating barriers to the right to health for these groups [71]. To move towards a truly inclusive healthcare system, it is essential to implement strategies that address these barriers from an intersectional perspective, promoting cultural and structural competence training for healthcare professionals and ensuring accessibility and adequacy in a genuinely racism-reflexive healthcare system. Furthermore, the inclusion of racialised people in the design and implementation of health research, as well as in health policies, is crucial to overcoming ethnocentric approaches and promoting meaningful progress. Only in this way will it be possible to transform institutional commitments into real changes that ensure equity in access to and quality of healthcare for all individuals, irrespective of their racial or ethnic background.

## Supplementary Information

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Supplementary Material 1

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**Author contributions**
YGR. Conception and design of the study, Analysis and interpretation of data, Drafting the article. NL. Design of the study, Analysis and interpretation of data, Drafting the article. ERA. Design of the study, Analysis and interpretation of data. All authors critically have read and approved the submitted version manuscript. All authors have agreed both to be personally accountable for the author's own contributions and to ensure that questions related to the accuracy or integrity of any part of the work, even ones in which the author was not personally involved, are appropriately investigated, resolved, and the resolution documented in the literature.

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**Data availability**
The datasets generated and/or analysed during the current study are available in the Zenodo repository, https://doi.org/10.5281/zenodo.15173507.

## Declarations

**Ethics approval and consent to participate**
Not applicable.

**Consent for publication**
Not applicable.

**Competing interests**
The authors declare no competing interests.

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